

H.R. 1 Medicaid Cuts: Coverage Loss, Life Expectancy, Hospital Assessment and Provider Shortage by Arizona Legislative District

Prepared September 4, 2026; provider-shortage section added September 6, 2026 · Statistical analysis with district-level allocation, mortality modeling, and independent audit

The bottom line

Arizona's Medicaid program, AHCCCS, is about to shrink under H.R. 1 (P.L. 119-21). Using the strongest available projections and the peer-reviewed evidence on what losing coverage does to mortality, we estimate that:

- **About 219,000 Arizonans lose AHCCCS coverage in the first full year** of the January 1, 2027 work requirement and six-month renewals, plus the October 1, 2026 immigrant-eligibility cut (range 134,000–280,000).
- **About 327,000 fewer Arizonans are covered by 2034** when every provision is fully in force (range 248,000–407,000). Roughly 248,000 of them become uninsured; KFF's estimate is higher, at 290,000.
- **About 545 additional deaths a year** once the coverage losses settle in (range 225–1,100), concentrated in working-age adults.
- **Statewide life expectancy at birth falls by about 0.17 years — roughly two months** (range 0.07–0.34 years). In the hardest-hit districts the drop is 3 to 3.6 months.
- **Every one of the 30 legislative districts is affected.** The heaviest losses fall in west and south Phoenix (LDs 26, 24, 11, 22), south Tucson (LDs 21, 20), Yuma (LD 23) and northern Arizona (LDs 6, 30). Even the least-affected district (LD 3, north Scottsdale) loses about 3,200 people from coverage.

Federal Medicaid funding to Arizona falls by roughly \$34.5 billion over ten years (KFF/CBO, range \$25.9–43.1B); JLBC puts the hospital-assessment loss alone at \$1.1 billion a year by FY 2032. Arizona's expansion "trigger" law is not activated by the enacted bill.

District-by-district estimates

Losses are people losing AHCCCS enrollment. "First full year" is the Urban Institute's 2028 projection for the work requirement and six-month renewals (start date Jan. 1, 2027) plus AHCCCS's count of Oct. 2026 immigrant-provision notices; "full phase-in" is RAND's 2034 projection. Deaths and life expectancy use the full phase-in central case. Life expectancy is period life expectancy at birth, baseline anchored to the latest (2020–2022) county data.

LD	Main county	Lose AHCCCS, first full year	Lose AHCCCS, full phase-in (2034)	Full phase-in range	Share of adults 19–64	Added deaths per year	Life expectancy now	Life expectancy with H.R. 1	Change (months)
1	Yavapai	5,800	9,000	6,800–11,200	7.4%	19	76.5	76.3	-1.9

LD	Main county	Lose AHCCCS, first full year	Lose AHCCCS, full phase-in (2034)	Full phase-in range	Share of adults 19–64	Added deaths per year	Life expectancy now	Life expectancy with H.R. 1	Change (months)
2	Maricopa	8,400	11,700	8,900–14,600	7.3%	19	76.9	76.8	-1.9
3	Maricopa	2,500	3,200	2,400–4,000	2.5%	6	81.1	81.1	-0.7
4	Maricopa	4,300	5,900	4,400–7,300	4.0%	11	80.0	79.9	-1.2
5	Maricopa	8,800	13,200	10,000–16,400	8.3%	22	75.1	74.9	-2.1
6	Coconino	8,200	11,800	9,000–14,700	9.4%	19	68.8	68.6	-1.8
7	Pinal	6,200	9,400	7,100–11,700	7.3%	18	74.8	74.6	-1.8
8	Maricopa	7,900	11,300	8,600–14,000	6.2%	17	77.0	76.8	-1.9
9	Maricopa	8,400	12,900	9,800–16,100	8.8%	20	76.0	75.8	-2.3
10	Maricopa	5,200	7,600	5,800–9,500	6.0%	15	77.7	77.6	-1.6
11	Maricopa	10,100	15,400	11,700–19,200	10.7%	24	75.0	74.8	-2.7
12	Maricopa	4,300	6,100	4,600–7,600	3.9%	10	79.3	79.2	-1.1
13	Maricopa	4,200	5,700	4,300–7,100	4.1%	9	79.0	78.9	-1.1
14	Maricopa	3,700	5,100	3,900–6,300	3.4%	8	78.9	78.8	-0.9
15	Maricopa	4,200	6,500	4,900–8,000	4.3%	11	79.2	79.1	-1.3
16	Pinal	7,300	11,100	8,400–13,800	7.8%	19	77.1	76.9	-2.1
17	Pima	4,100	6,300	4,700–7,800	4.9%	11	78.7	78.6	-1.3
18	Pima	7,400	11,100	8,400–13,700	8.0%	19	76.5	76.3	-2.1
19	Cochise	6,800	10,700	8,100–13,200	9.9%	19	76.7	76.5	-2.5
20	Pima	10,400	16,300	12,300–20,200	10.8%	26	75.0	74.7	-2.9
21	Pima	11,100	17,300	13,100–21,500	12.7%	26	75.8	75.5	-3.1
22	Maricopa	9,400	14,500	11,000–18,100	9.1%	22	76.2	76.0	-2.4
23	Yuma	10,000	15,700	11,900–19,500	11.7%	24	77.5	77.3	-3.0

LD	Main county	Lose AHCCCS, first full year	Lose AHCCCS, full phase-in (2034)	Full phase-in range	Share of adults 19–64	Added deaths per year	Life expectancy now	Life expectancy with H.R. 1	Change (months)
24	Maricopa	12,800	19,400	14,700–24,100	13.8%	28	75.2	74.9	-3.4
25	Maricopa	6,900	10,200	7,700–12,700	7.5%	17	77.9	77.7	-2.0
26	Maricopa	15,000	21,300	16,200–26,500	14.5%	32	75.0	74.7	-3.6
27	Maricopa	8,300	12,200	9,200–15,100	8.2%	20	77.3	77.2	-2.1
28	Maricopa	3,500	4,800	3,600–6,000	4.5%	10	78.1	78.0	-1.2
29	Maricopa	6,000	9,000	6,800–11,200	6.5%	16	77.5	77.3	-1.8
30	Mohave	8,200	12,700	9,600–15,700	10.7%	26	72.8	72.6	-2.4
Arizona statewide		219,000	327,000	248,000–407,000	7.8%	545	76.8	76.6	-2.0

Central estimates; rounded. Range = bounding envelope combining the low and high ends of the coverage-loss and mortality-effect evidence. Full detail, sources and formulas are in the accompanying Excel workbook.

How we got here

- **Statewide anchors.** First full year: Urban Institute (Mar. 2026) — Arizona expansion enrollment falls 123,000 to 246,000 in 2028 depending on how well the state implements (central 197,000), plus 22,263 AHCCCS advance notices for the Oct. 1, 2026 immigrant-eligibility change. Full phase-in: RAND (June 2026) — 327,400 fewer AHCCCS enrollees in 2034, of which work requirements 198,800, provider-tax retrenchment 94,200, six-month renewals 65,700. The range applies KFF/CBO's $\pm 24\%$ Arizona uncertainty band. JLBC's own estimate (171,000 by FY 2030 from the two eligibility provisions) sits inside this range.
- **District allocation.** AHCCCS does not publish enrollment by legislative district or ZIP code, so we allocated the statewide figures using Census ACS 2020–2024 tract data on adults 19–64 with Medicaid (excluding Medicare dual-eligibles and adults reporting a disability), mapped to the 2022 IRC districts block by block. American Indians are exempt from the work requirement by statute and were removed from that component. The allocation was calibrated to AHCCCS's actual expansion enrollment by congressional district (factors 0.90–1.12, a close fit) and to county enrollment.
- **Deaths.** 0.76 uninsured per enrollee lost (CBO). Then 1.2 / 2.2 / 3.6 additional deaths per year per 1,000 uninsured — from Sommers, Long & Baicker (Annals of Internal Medicine 2014), the Harvard/CUNY projection of H.R. 1 (Gaffney et al., Annals 2025) and Sommers (Am. J. Health Econ. 2017). Consistent with the Yale/Penn projection of 42,500 deaths a year nationally.
- **Life expectancy.** CDC census-tract life expectancy (USALEEP) aggregated to districts and ZIP codes, shifted to the latest county figures (statewide 76.7, matching NCHS). New life expectancy recomputed with the Arizona 2022 life table after adding the district's additional deaths at ages 19–64.
- **Triple-checked.** An independent audit re-derived every district figure from the raw data with separate code, confirmed all 13 spot-checked source claims against the live sources, and flagged two methodological problems (the American Indian exemption and district-scaled death rates) that were corrected before this memo was finalized.

The hospital assessment step-down: who loses what

Arizona's hospital assessment sits at the federal ceiling — effectively 6.0% of hospital net patient revenue — and it is the engine of the state's Medicaid financing. In SFY 2025 it collected **\$1.36 billion** from hospitals and returned **\$4.32 billion** to 134 Arizona facilities: \$1.50 billion in coverage payments for the adults it insures and \$2.83 billion in HEALTHII directed payments. Every assessment dollar comes back as \$3.19. H.R. 1 (Sec. 71115) does not repeal the assessment; it lowers the ceiling half a point a year from October 1, 2027 (5.5%) until it reaches **3.5% in FFY 2032**, then freezes it there. Because Arizona has no unused room under the cap, every step bites immediately.

Federal fiscal year	Ceiling	Assessment not collected	Payments lost to hospitals	Net loss to hospitals
FFY 2028	5.5%	\$112.9M	\$360.4M	\$247.5M
FFY 2029	5.0%	\$225.9M	\$720.8M	\$494.9M
FFY 2030	4.5%	\$338.8M	\$1,081.1M	\$742.4M
FFY 2031	4.0%	\$451.7M	\$1,441.5M	\$989.8M
FFY 2032 and each year after	3.5%	\$564.6M	\$1,801.9M	\$1,237.3M
Ten years, FFY 2028–2037		\$4.52B	\$14.42B	\$9.90B

Method: each facility's SFY 2025 assessment paid and assessment-funded payments received (AHCCCS, SFY 2025 Hospital Assessment Report, June 1, 2026) scaled by the share of the 6.0% rate removed, federal match held constant, constant dollars — the same method and totals as the CSPA one-pager. "Payments lost" is the reduction in Medicaid revenue a hospital receives; "net loss" subtracts the assessment it no longer pays. A projection, not an AHCCCS figure.

By hospital system

The step-down is not spread evenly. Because payments track assessment paid plus federal match, the systems with the largest Medicaid volumes carry the largest cuts. **Banner Health alone is looking at a \$657 million annual reduction in Medicaid payments once the rate reaches 3.5% — \$461 million net of the assessment it no longer pays, and \$3.7 billion net over ten years.** Dignity Health, HonorHealth and Abrazo each lose \$146–181 million a year in payments; Valleywise, the county safety-net system, loses \$66 million with no offsetting assessment savings because it pays none.

Hospital system	Facilities	SFY 2025 payments	Payments lost per year at 3.5%	Net loss per year	Ten-year net loss
Banner Health	18	\$1.58B	\$657.3M	\$460.8M	\$3.69B
Dignity Health	8	\$435.3M	\$181.4M	\$88.6M	\$708.9M
HonorHealth	9	\$393.3M	\$163.9M	\$74.5M	\$595.9M
Abrazo Health Care	8	\$350.3M	\$146.0M	\$88.5M	\$707.9M
IHS (IHS/tribal — 100% federal match)	10	\$173.8M	\$72.4M	\$72.4M	\$579.3M
TMC Healthcare	3	\$167.9M	\$70.0M	\$49.3M	\$394.4M
Onvida Health (Yuma Regional Medical Center)	1	\$165.3M	\$68.9M	\$50.0M	\$400.1M
Valleywise Health	1	\$159.6M	\$66.5M	\$66.5M	\$531.9M
Community Health Systems	5	\$117.1M	\$48.8M	\$21.5M	\$172.2M
Phoenix Children's Hospital	1	\$110.6M	\$46.1M	\$41.3M	\$330.3M
Lifepoint Health	7	\$109.8M	\$45.7M	\$31.1M	\$248.5M
Northern Arizona	2	\$102.0M	\$42.5M	\$27.4M	\$219.3M
Kingman Regional Medical Center	1	\$69.1M	\$28.8M	\$19.9M	\$159.4M

Hospital system	Facilities	SFY 2025 payments	Payments lost per year at 3.5%	Net loss per year	Ten-year net loss
Summit Healthcare	1	\$58.3M	\$24.3M	\$16.9M	\$135.0M
All other systems (37 systems)	59	\$334.7M	\$139.5M	\$128.6M	\$1.03B
Statewide	134	\$4.32B	\$1.80B	\$1.24B	\$9.90B

The twelve most exposed facilities

Facility	System	LD	SFY 2025 payments	Payments lost per year	Net loss per year
Banner – University Medical Center Phoenix	Banner Health	5	\$306.3M	\$127.6M	\$100.0M
Banner – University Medical Center Tucson	Banner Health	20	\$298.3M	\$124.3M	\$99.9M
Banner Desert Medical Center	Banner Health	9	\$236.3M	\$98.5M	\$79.6M
St. Joseph's Hospital and Medical Center (Phoenix)	Dignity Health	5	\$213.8M	\$89.1M	\$56.9M
Banner Thunderbird Medical Center	Banner Health	27	\$186.0M	\$77.5M	\$57.3M
Onvida Health (Yuma Regional Medical Center)	Onvida Health (Yuma Regional Medical Center)	25	\$165.3M	\$68.9M	\$50.0M
Tucson Medical Center	TMC Healthcare	18	\$160.5M	\$66.9M	\$46.7M
Valleywise Health Medical Center	Valleywise Health	5	\$159.6M	\$66.5M	\$66.5M
HonorHealth John C. Lincoln Medical Center	HonorHealth	5	\$119.4M	\$49.7M	\$29.3M
Banner Estrella Medical Center	Banner Health	22	\$114.6M	\$47.8M	\$30.2M
Phoenix Children's Hospital	Phoenix Children's Hospital	5	\$110.6M	\$46.1M	\$41.3M
Banner Gateway Medical Center	Banner Health	14	\$95.7M	\$39.9M	\$19.2M

By legislative district

Each facility was assigned to the district where it physically sits (street address geocoded to a 2020 census block, then to the 2022 IRC map). Hospitals draw patients and staff from many districts, so read these as where the buildings, payrolls and cuts land. LD 5 (central Phoenix) absorbs a quarter of the statewide hit because it holds Banner – University Medical Center Phoenix, St. Joseph's, Valleywise, Phoenix Children's, John C. Lincoln and Phoenix Indian Medical Center; LD 20 (Banner – UMC Tucson) and LD 18 (TMC, Carondelet St. Joseph's and seven others) follow. Every district has at least one participating facility.

LD	Facilities	SFY 2025 payments	Payments lost per year at 3.5%	Net loss per year	Ten-year net loss
1	5	\$80.1M	\$33.4M	\$15.1M	\$120.6M
2	2	\$81.6M	\$34.0M	\$20.8M	\$166.8M
3	3	\$26.2M	\$10.9M	–\$0.6M	–\$4.7M
4	3	\$105.5M	\$44.0M	\$20.6M	\$164.7M
5	12	\$985.7M	\$410.7M	\$324.1M	\$2.59B
6	13	\$224.6M	\$93.6M	\$82.2M	\$657.7M
7	5	\$102.0M	\$42.5M	\$32.4M	\$259.6M
8	6	\$108.4M	\$45.2M	\$25.6M	\$205.1M
9	2	\$237.3M	\$98.9M	\$80.0M	\$639.7M

LD	Facilities	SFY 2025 payments	Payments lost per year at 3.5%	Net loss per year	Ten-year net loss
10	6	\$86.4M	\$36.0M	\$15.1M	\$120.5M
11	4	\$26.3M	\$11.0M	\$7.2M	\$57.3M
12	3	\$30.7M	\$12.8M	\$11.7M	\$93.4M
13	3	\$109.1M	\$45.5M	\$18.4M	\$147.4M
14	5	\$148.3M	\$61.8M	\$25.4M	\$203.6M
15	3	\$39.7M	\$16.6M	\$6.8M	\$54.5M
16	3	\$76.4M	\$31.9M	\$26.7M	\$213.4M
17	2	\$20.0M	\$8.3M	\$2.4M	\$19.0M
18	10	\$328.8M	\$137.0M	\$90.4M	\$723.5M
19	4	\$53.8M	\$22.4M	\$16.1M	\$129.1M
20	2	\$351.1M	\$146.3M	\$114.8M	\$918.1M
21	4	\$128.8M	\$53.7M	\$39.9M	\$319.2M
22	3	\$133.5M	\$55.6M	\$36.9M	\$295.4M
23	3	\$73.7M	\$30.7M	\$19.0M	\$151.9M
24	1	\$17.1M	\$7.1M	\$6.5M	\$52.2M
25	4	\$167.7M	\$69.9M	\$51.0M	\$408.2M
26	2	\$49.4M	\$20.6M	\$15.2M	\$121.4M
27	4	\$268.0M	\$111.7M	\$75.5M	\$604.3M
28	7	\$112.8M	\$47.0M	\$16.8M	\$134.3M
29	1	\$3.7M	\$1.5M	\$1.5M	\$12.3M
30	9	\$147.7M	\$61.5M	\$39.6M	\$317.1M
Arizona	134	\$4.32B	\$1.80B	\$1.24B	\$9.90B

Read before quoting

- **Two ways to count the cut.** This section spreads the reduction across both funds in proportion (the one-pager method). JLBC's January 2026 analysis assumes AHCCCS protects the coverage match first and concentrates the cut in HEALTHII directed payments; on that assumption the statewide loss is smaller in early years (\$1.1B a year by FY 2032 against FY 2026 levels, \$2.1B against the pre-law trajectory) and falls harder on the hospitals with the largest HEALTHII payments. The direction and the ranking of systems are the same either way.
- **IHS and tribal facilities.** 11 Indian Health Service and tribal (638) facilities appear in AHCCCS's report with \$199.7M in payments and no assessment paid. Medicaid services at these facilities are matched at 100% federal funds and are not financed by the assessment, so their \$83.2M is included in the statewide total only to tie to the one-pager; excluding them, the net loss to Arizona's assessment-paying hospital system is \$1.15B a year.
- **Assessment-funded payments only.** These figures cover the two assessment-funded streams AHCCCS reports. They do not include the separate H.R. 1 cap on state-directed payments (HEALTHII is also exposed to that provision from January 2028), Medicaid revenue lost as 220,000–330,000 Arizonans lose coverage, or the effect on physician and dental payments that share the HCIF. The true hit to hospitals is larger than shown here.
- **Base year.** SFY 2025 flows in constant dollars; JLBC projects the assessment base growing about 5% a year, which would raise every figure proportionally.

- Two facilities (HonorHealth Thompson Peak, Dignity Arizona General – Mesa) paid more in assessment than they received and show a small net gain from the step-down.

The provider shortage the cuts land on

Every Arizonan who loses AHCCCS coverage will be looking for care in a state that already ranks **42nd in primary care physicians per capita** — 80 per 100,000 residents against 94 nationally (UA Center for Rural Health, Feb. 2025). HRSA has designated **227 primary care shortage areas** in Arizona that would need **469 more full-time primary care physicians** to be removed, plus 212 mental health and 210 dental shortage areas; all 15 counties contain at least one. The state meets **35.4% of its primary care need** (AzCRH, Dec. 2024). Hospitals absorbing a \$1.24 billion assessment hit and a quarter-million newly uninsured patients will do so with fewer clinicians per resident than almost any state.

Where the doctors are — by legislative district

The only sub-county administrative count of clinicians is CMS's Medicare enrollment file (Doctors and Clinicians, refreshed July 31, 2026), which records every Medicare-enrolled clinician's practice address. We assigned 35,922 Arizona clinicians to districts by that address (clinicians with several offices split equally). The statewide result — 60 primary care physicians per 100,000 — sits beside HRSA's 63.9, which confirms the mapping. The distribution is not: practices cluster around hospitals, so central Phoenix and Tucson read high while west Phoenix and the West Valley have a fraction of the state's already-thin coverage. **LD 24 (Maryvale) has 9 primary care physicians per 100,000 residents; LD 25 has 18; LD 26 has 25.** Those three districts are also among the five losing the most coverage.

LD	Main county	Primary care physicians per 100,000	All physicians per 100,000	Psychiatrists per 100,000	NPs + PAs per 100,000
24	Maricopa	9	21	6.2	33
25	Maricopa	18	54	1.3	57
26	Maricopa	25	60	6.2	65
15	Maricopa	28	69	0.1	92
11	Maricopa	30	123	4.9	97
22	Maricopa	32	102	1.8	91
1	Yavapai	35	144	2.7	137
19	Cochise	35	119	6.9	87
29	Maricopa	38	96	2.0	123
7	Pinal	39	163	3.6	91
13	Maricopa	41	149	3.2	114
17	Pima	44	135	3.2	90
14	Maricopa	44	204	6.1	198
2	Maricopa	46	174	4.4	129
16	Pinal	48	122	5.5	103
21	Pima	49	140	6.6	75
10	Maricopa	49	178	1.4	145
23	Yuma	51	202	2.0	123
9	Maricopa	56	201	11.8	130
8	Maricopa	58	243	23.5	216

LD	Main county	Primary care physicians per 100,000	All physicians per 100,000	Psychiatrists per 100,000	NPs + PAs per 100,000
30	Mohave	61	252	3.0	126
12	Maricopa	61	189	14.0	144
3	Maricopa	73	489	6.2	246
28	Maricopa	78	229	5.8	171
27	Maricopa	90	285	6.4	195
20	Pima	92	324	12.6	140
6	Coconino	110	344	12.3	145
18	Pima	111	417	14.6	243
4	Maricopa	111	500	14.2	302
5	Maricopa	232	858	24.7	424
Arizona		60	220	7.3	145

Medicare-enrolled clinicians by practice location (CMS). Primary care = family practice, internal medicine, general practice, pediatrics, geriatrics (MD/DO). Excludes clinicians who do not bill Medicare — many pediatric and behavioral-health practices — and most IHS clinicians, so tribal-area districts are understated. These are counts of where practices sit, not where residents receive care.

By county — the federal record

County	Primary care physicians per 100,000 (2023)	All physicians per 100,000	Psychiatrists per 100,000	Primary care HPSAs	Primary care FTEs short	Mental health HPSAs
Pinal	17.3	42	1.7	26	71.9	24
La Paz	23.9	84	0.0	10	3.6	10
Santa Cruz	38.7	88	0.0	3	5.9	3
Apache	40.0	89	1.5	13	7.5	14
Yuma	41.7	151	2.3	7	24.9	7
Yavapai	47.8	198	10.0	10	14.8	9
Graham	48.1	84	0.0	9	8.3	9
Gila	48.1	130	3.7	12	6.2	12
Mohave	48.7	174	3.1	12	26.4	11
Cochise	57.0	143	1.6	15	12.0	15
Navajo	58.6	143	1.8	21	10.2	21
Greenlee	64.0	75	0.0	1	2.7	1
Maricopa	67.3	304	8.6	50	188.6	37
Coconino	81.7	297	13.2	17	15.1	16
Pima	85.3	436	17.4	29	70.8	29

HRSA Area Health Resources File 2024–25 (AMA/AOA 2023; primary care = patient-care physicians excluding residents) and HRSA HPSA designations in force Sept. 5, 2026 (Designated status; multi-county HPSAs counted in each county with FTE shortage split equally). Pinal County — 514,000 people, 17 primary care physicians per 100,000 — is the most under-served county of its size in the state.

The hardest-hit Primary Care Areas

The Arizona Department of Health Services divides the state into 126 Primary Care Areas. Its 2021 profiles (the latest published) show 747 residents per primary care provider statewide; the worst areas with 5,000 or more residents:

Primary Care Area	County	Population	Primary care providers	Residents per provider
Laveen Village	Maricopa	79,094	3	26,365
Vail	Pima	24,370	2	12,185
Valencia West	Pima	21,554	2	10,777
Quartzsite	La Paz	9,687	1	9,687
Rio Rico	Santa Cruz	27,462	3	9,154
Coolidge	Pinal	18,306	2	9,153
Flowing Wells	Pima	16,993	2	8,497
Apache Junction	Maricopa & Pinal	49,540	6	8,257

The pipeline

Occupation	Arizona employment, May 2025	Location quotient (1.0 = U.S. average)	Median annual wage	Projected change 2024–34	Average annual openings
Family Medicine Physicians	2,620	1.17	\$292,760	+14.4%	170
General Internal Medicine Physicians	2,570	1.84	\$259,320	+10.7%	70
Pediatricians, General	1,230	1.50	\$233,950	+13.6%	40
Psychiatrists	380	0.64	\$249,720	+17.2%	20
Physicians, All Other	6,800	0.95	\$312,490	+14.1%	220
Nurse Practitioners	7,220	1.07	\$134,420	+54.4%	860
Physician Assistants	3,750	1.11	\$134,650	+32.2%	320
Registered Nurses	73,150	1.04	\$99,500	+18.6%	4,990
Licensed Practical and Licensed Vocational Nurses	6,530	0.48	\$77,070	+15.9%	550
Nursing Assistants	20,320	0.67	\$44,780	+17.4%	3,180
Mental Health and Substance Abuse Social Workers	3,690	1.33	\$46,380	+23.9%	480
Dentists, General	3,470	1.34	\$194,180	+14.2%	140
Dental Hygienists	4,530	0.98	\$101,010	+16.2%	420
Pharmacists	6,900	1.03	\$146,930	+17.9%	460

BLS Occupational Employment and Wage Statistics, May 2025, Arizona statewide ("n/p" = not published by BLS); Arizona Office of Economic Opportunity long-term projections 2024–2034 via Projections Central. A location quotient below 1.0 means Arizona employs proportionally fewer of that occupation than the nation: psychiatrists (0.64), nursing assistants (0.67) and LPNs (0.48) are the clearest gaps. Growth projections assume current funding; H.R. 1 removes roughly \$1.8 billion a year of the hospital revenue that pays those salaries. Job-postings data (Indeed, Lightcast) are proprietary and change daily, so they are not used here.

What to be careful about when using these numbers

- District figures are **allocations** of statewide projections, not counts; treat them as best estimates with ranges.

- Exemptions for parents of young children and the medically frail cannot be located geographically; family-heavy districts may be modestly overstated.
- About a quarter of the 2034 loss is RAND's assumption that Arizona trims optional eligibility as provider-tax revenue falls; the workbook shows results with and without it.
- Mortality evidence comes from studies of people **gaining** coverage; every published H.R. 1 death projection assumes the effect runs in reverse. Effects build over three to four years, so first-year deaths will be smaller.
- Life-expectancy baselines reflect 2020–2022 mortality, which is pandemic-depressed, especially in Apache, Navajo and Gila counties (LDs 6 and 7); the *change* is what matters, and it barely depends on the baseline level.
- Not included: ACA marketplace losses (Arizona plan selections already fell ~66,000 for 2026), hospital and clinic closures, provider payment cuts, and SNAP-related health effects. These push the true impact higher, not lower.

Key sources

- [RAND, State-Level Impacts of Key Medicaid Provisions in the OBBBA \(v2, June 2026\)](#)
- [KFF, How Will the 2025 Reconciliation Law Affect the Uninsured Rate in Each State? \(Aug. 2025\)](#)
- [Urban Institute, Projected Reductions in Medicaid Expansion Enrollment Under OBBBA \(Mar. 2026\)](#)
- [JLBC Staff, Analysis of H.R. 1 on the Arizona State Budget \(Jan. 2026\)](#)
- [AHCCCS, SFY 2025 Hospital Assessment Report \(June 1, 2026\)](#); [AHCCCS, Hospital Assessment page](#)
- [AHCCCS, Population Statistics \(Aug. 2026\)](#) and [HMA/AHCCCS, Data to Inform by Congressional District](#)
- [AzPHA, Arizona's First H.R. 1 Medicaid Cuts Arrive October 1 \(Aug. 31, 2026\)](#)
- [CBO, Supplemental Estimate of P.L. 119-21 Medicaid Provisions \(Oct. 2025\)](#)
- [CMS, Doctors and Clinicians National Downloadable File](#); [HRSA, HPSA data downloads](#); [HRSA, Area Health Resources File](#); [UA Center for Rural Health, Quantifying Arizona's Primary Care Shortage \(Dec. 2024\)](#); [Derksen, Primary Care Physician Workforce Summit \(Feb. 2025\)](#); [ADHS, Primary Care Area statistical profiles](#); [BLS, OEWS May 2025](#); [Arizona OEO employment projections](#)
- [CDC/NCHS, USALEEP census-tract life expectancy](#); [NCHS, U.S. State Life Tables 2022](#); [County Health Rankings 2025](#)
- [Sommers, Long & Baicker, Annals of Internal Medicine 2014](#); [Gaffney, Himmelstein & Woolhandler, Annals 2025](#); [Sommers, Am. J. Health Econ. 2017](#); [Wyse & Meyer, NBER 2025](#); [Penn LDI / Yale mortality memo](#)